

WHO BENEFITS FROM A CLOSE-KNIT BLOCK?

Neighborhood Cohesion Pays Off Unevenly for Self-Rated Health Across Racial and Ethnic Groups

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INTRODUCTION

Trusting your neighbors is treated as a health resource, and communities are often urged to build social ties as a way to close health gaps. But a resource only helps if the conditions around it let it work. This study asks whether the health benefit of neighborhood cohesion is shared equally across racial and ethnic groups.

KEY FINDINGS

- Each one-point gain in cohesion was linked to **6% lower odds** of poor self-rated health after full adjustment (OR = 0.94).
- That benefit was **not shared equally**: the cohesion-by-race interaction was significant ($p < 0.001$).
- The association was **weaker** for Hispanic adults (OR = 0.96) and non-Hispanic Black adults (OR = 0.96) than for non-Hispanic White adults (OR = 0.93), roughly half the benefit for Black adults.
- Across the cohesion range, predicted poor health fell from **7.0% to 2.9%** for White adults but only **6.2% to 3.9%** for Black adults, leaving them worse off at the top of the scale.
- The pattern held under multiple imputation, a binary cohesion measure, an ordinal outcome, alternative age windows, and adjustment for home ownership and health care access.

WHY IT MATTERS

- Cohesion is a real correlate of health, but **not an equal one**. Treating race as a covariate rather than a modifier hides this.
- Social ties alone are unlikely to close health gaps while the material and institutional conditions around them stay unequal.
- Building neighborly connection may work better when paired with investment in local health infrastructure, food access, and community health workers.
- Addressing segregation and concentrated poverty may be what lets cohesion pay off for everyone.

RESEARCH DESIGN

- Analyzed **117,126 adults aged 25 to 65** from the National Health Interview Survey (2013–2018).
- Measured perceived neighborhood cohesion (4-item scale, 4 to 16, $\alpha = 0.89$):
 - neighbors help each other out
 - people I can count on
 - people can be trusted
 - a close-knit neighborhood
- Modeled poor self-rated health (fair or poor vs. good or better) with survey-weighted logistic regression.
- Tested moderation by race and ethnicity (confirmatory), and by sex and neighborhood tenure (exploratory), with joint Wald tests and Holm adjustment.
- Confirmed the scale works the same way across groups (scalar invariance in multi-group CFA), so the differences are not a measurement artifact.

MAIN THEMES

The same neighborhood trust buys *less* health for Hispanic and Black adults than for White adults. This extends the minorities' diminished returns framework from individual resources such as education and income to a shared neighborhood one, and cautions against treating social connection as a substitute for structural investment.

FOR MORE

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<https://rdcu.be/fB0KP>

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